DOMAIN CONTENT · BATCH 01 OF 14

Two voices, *one science.*

SITE 01

Health Decoded

KYG.ai · A longform journal on genomic wellness

VOICE NOTE

Warm magazine. Six to twelve minute reads. The writer is on the reader's side, not above them. Each piece earns its conclusion through evidence, not assertion. Citations appear in the prose. Anecdotes are short and serve the science.

01 · HOMEPAGE

Homepage copy

Replaces all placeholder text in the existing UI. Section labels match the HTML structure provided.

Hero · Eyebrow label

> A journal on what your DNA is trying to tell you.

Hero · Headline

> Genomic wellness, decoded for real life.

Stays as-is. The existing line works — it is concrete, it promises plain language, it does not oversell. Resist any urge to make it grander.

Hero · Subheadline

Bite-sized reads on how your DNA shapes nutrition, fitness, sleep, recovery, and the long arc of preventive care. Written for South Asian biology, written for the everyday. No jargon, no fad science, no diet wars. Just what the research says, plainly, with what to do about it.

Hero · Primary CTA

[Start reading]

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Hero · Secondary link

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Editor's Pick · Strap

EDITOR'S PICK · GENOMIC SCIENCE · 12 MIN READ

Editor's Pick · Headline

> Why one-size-fits-all health advice was never built for Indian biology.

Editor's Pick · Standfirst

South Asians develop heart disease nearly a decade earlier than Caucasians. We are the world's largest producer of dairy, and most of us cannot digest it. The vitamin D deficiency rates in urban Indian cohorts cross ninety percent. None of this is in the standard health pamphlet. This is why the standard health pamphlet keeps failing us.

Editor's Pick · Byline

By the editors · Published today · Filed under Genomic Science

Editor's Pick · CTA

Read the full article →

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Category filter chips

Keep the seven existing categories:

—All

—Genomic Science

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Latest from the journal · Section heading

Latest from the journal

Showing 9 of 84 articles

Article card copy · Nine cards

Each card has a category label, read time, headline, two-sentence dek, author chip. Below: nine cards, written to the cream-magazine voice. Authors are placeholder bylines — replace with your real contributor list.

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Card 01 · Nutrition · 6 min

Why your "healthy" diet might not be working for you

The same plate of dal, rice, and sabzi will spike one person's blood sugar and barely move another's. The reason sits in a handful of genes most of us have never heard of. Here is what the research actually says about personalised nutrition, and what to ignore.

By Ananya R. · 3 days ago

Card 02 · Genomic Science · 9 min

The science of genetic testing, explained without the jargon

SNPs. Polygenic scores. Variants of uncertain significance. The language of genomics is built to keep most people out of the conversation. We translate. What a test can tell you, what it cannot, and how to read a report without spiralling.

By Dr. Vikram K. · 5 days ago

Card 03 · Fitness · 7 min

Why two people respond differently to the same workout

One person gains muscle in six weeks of lifting. The other does the same programme and gets nothing. Genetics explains roughly half of that gap. Here is what the ACTN3, ACE, and PPARGC1A variants are actually doing inside the gym.

By Priya S. · 1 week ago

Card 04 · Preventive Care · 11 min

Building a preventive health routine in your 30s and 40s

The decade between thirty-five and forty-five is when most lifestyle conditions begin their slow accumulation, often without symptoms. What to track, what to test, and what to safely ignore as you enter the high-leverage years.

By Dr. Riya M. · 1 week ago

Card 05 · Lifestyle · 5 min

The genetics of sleep: why some of us need six hours and others nine

Your chronotype is not a personality quirk and it is not a moral failing. It is largely written in your PER3 and CRY1 genes. Here is how to work with the schedule your biology prefers, instead of against it.

By Sneha K. · 2 weeks ago

Card 06 · Senior Wellness · 8 min

Aging well in India: what the latest research actually says

Beyond the supplement aisle and the ten-thousand-step myth, the evidence-backed habits that move the needle on healthspan after fifty. The protein gap. The grip strength signal. The two tests every Indian over fifty should ask for.

By Dr. Aruna G. · 2 weeks ago

Card 07 · Nutrition · 6 min

Spice as medicine: the science behind your masala dabba

Turmeric is having a moment in the West. Indians have had it on the stove for three thousand years. We look at what the research actually shows about haldi, methi, jeera, and ajwain — separating the documented benefits from the wellness-industry mythology.

By Neha B. · 3 weeks ago

Card 08 · Genomic Science · 10 min

Inherited but not inevitable: how epigenetics rewrites the rules

The genes you were born with are not a sentence. They are an instruction set, and the instructions can be turned up or down by diet, sleep, stress, and movement. Here is what the new science of epigenetics is telling us about the gap between genetic risk and lived outcome.

By Dr. Manas K. · 3 weeks ago

Card 09 · Lifestyle · 4 min

Stress, cortisol, and the genes that decide your baseline

Why the same workload breaks one person and energises another. Your COMT and FKBP5 variants set the speed at which your nervous system clears stress hormones. For some people, recovery from a hard week takes a weekend. For others, it takes a fortnight. Here is what to do if you are the second type.

By Tanya R. · 4 weeks ago

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Footer · Mission line

A journal on genomics, wellness, and preventive health — written for the everyday, grounded in the research, built for South Asian biology.

02 · FLAGSHIP ARTICLE

Flagship · Why one-size-fits-all health advice was never built for Indian biology

~ 2,000 words · 12 minute read · Filed under Genomic Science · Byline: editorial.

STRUCTURE

Open with a scene that lands the question. Move into the three best-documented examples (lactose, vitamin D, cardiovascular onset). Bring in the genetic mechanism in plain language. End with what to actually do about it, and only then place the affiliate CTA — earned, in context, and reversible.

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Standfirst

South Asians develop heart disease nearly a decade earlier than Caucasians. We are the world's largest producer of dairy, and most of us cannot digest it past childhood. The vitamin D deficiency rates in urban Indian cohorts cross ninety percent. None of this is in the standard health pamphlet. This is why the standard health pamphlet keeps failing us.

Body copy

There is a moment, somewhere in your late twenties or early thirties, when the wellness advice you have been ignoring all your life suddenly seems to apply to you. The doctor mentions cholesterol. A cousin gets diagnosed with diabetes. Your mother starts asking when you last had a blood test. You go to the internet, the internet sends you to a wellness influencer in California, and the wellness influencer tells you to try intermittent fasting, switch to oat milk, take a vitamin D3 supplement of 1,000 IU, and walk ten thousand steps a day.

Most of that advice was developed for a person who does not look like you, does not eat like you, and does not metabolise food the way your body does. Some of it will work. Some of it will not. A small fraction of it might actively hurt. The strange thing is, nobody is lying. The research the influencer is drawing on is real research. It just was not done on Indians.

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Three things the global handbook gets wrong about Indian bodies

Take the simplest example. The 2025 South Asian genome study published on bioRxiv pulled genetic data from roughly eight thousand individuals across India, Pakistan, and Bangladesh and looked at one of the most studied genes in human biology — LCT, the lactase gene. In most Northern European populations, somewhere between fifty and ninety percent of adults carry a variant that lets them digest milk into adulthood. In most South Asian populations, the figure is closer to twelve percent. The default state across the subcontinent is lactase non-persistence. We stopped making the enzyme around the time we were weaned, and never started again.

And yet India produces more dairy than any other country on earth. Doodh and dahi and paneer are not optional features of the Indian plate, they are foundational. The biology and the culture are out of phase, and most of us have been quietly negotiating with the consequences our whole lives — the bloating after the second chai, the discomfort after the heavy paneer dinner, the vague sense that something is off. Most Indians do not have lactose intolerance as a diagnosed condition. Most Indians have lactose intolerance as the unexamined background hum of their digestion.

The standard Western advice — "cut dairy if it bothers you, otherwise it's fine" — is built for a population where lactase persistence is the rule. For us, the rule is the opposite, and the advice needs to be reversed. Smaller portions. Fermented forms first (dahi and chaas before milk). The right kind of paneer, in the right quantity, at the right time of day. Or, if the data says so, a clean switch.

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Move to vitamin D. The studies in urban Asian Indian cohorts are not subtle. Mean serum 25(OH)D levels in one well-cited Indian study sat at around 9.8 nanograms per millilitre, which is squarely in the deficient range, and ninety-four percent of the subjects met clinical deficiency criteria. This is not a small effect in a small sample. This is a feature of how South Asian skin, melanin, and modern indoor life interact with the latitude of the subcontinent. Darker skin synthesises vitamin D more slowly. Air-conditioned office life cuts sun exposure further. The dietary sources of vitamin D in a typical Indian vegetarian diet are limited. The deficiency is not unusual. It is the median state.

The standard global supplementation advice — 1,000 IU per day, sometimes 2,000 — was calibrated against populations that are not starting from this baseline. For an Indian adult with deficient or insufficient status, that dose can be too low to move the needle for months. And the response itself is genetic. Variants in the VDR gene, particularly the FokI and BsmI polymorphisms, change how efficiently your cells respond to whatever vitamin D is in your blood. Two people taking the same supplement, eating the same dinner, and getting the same sun exposure can end up with very different downstream effects on bone density, immune function, and cardiovascular markers.

This is not a reason to take more. It is a reason to test, dose, retest, and personalise — which is exactly the protocol almost no general physician in India actually runs.

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The third example is the one we should have been talking about for thirty years. Cardiovascular disease in South Asians arrives roughly a decade earlier than it does in Western populations.

Multiple large studies — npj Cardiovascular Health published a 2025 systematic review, the BRAVE study out of Bangladesh, the ScienceDirect review on South Asian ASCVD — have converged on the same finding. Indians have their first myocardial infarction about ten years younger than Caucasians on average. The risk profile is also strange: thinner, lower BMI, often non-smoking patients are showing up in cardiac wards in their forties. The standard Framingham risk calculator, which was built on the Framingham, Massachusetts cohort beginning in 1948, systematically underestimates risk for us.

Some of this is lifestyle and some of it is genetic. FTO variants are associated with type 2 diabetes risk in South Asians in ways that do not always show up in European cohorts. Lipoprotein(a) levels — a less famous cardiovascular risk marker that conventional cholesterol panels do not measure — run higher in South Asian populations. The dyslipidaemia pattern is its own dialect: lower HDL, higher triglycerides, more abdominal fat at lower body weights. None of this means South Asians are doomed. It means the playbook has to be different.

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So what to actually do

The first move is the easiest one. Stop applying advice that was not built for you. The keto diet was tested largely on European and American populations. Intermittent fasting protocols have their best evidence in similar cohorts. The "eat more leafy greens" gospel, taken seriously, runs into iron absorption interactions specific to a vegetarian Indian diet. None of these are wrong. But none of them are guaranteed to work for the person reading this article, because the studies they came from were not run on the person reading this article.

The second move is to find out where your own biology actually is. The standard battery for an Indian adult who wants to take preventive care seriously runs something like this: an HbA1c and fasting insulin (not just fasting glucose), a full lipid panel including ApoB and Lp(a) if you can get them, a 25(OH)D level, a B12 level, a basic thyroid panel, and, increasingly, a wellness-focused DNA panel that covers the variants relevant to South Asian biology — LCT for lactose, MTHFR for folate metabolism, CYP1A2 for caffeine, FTO and TCF7L2 for metabolic risk, VDR for vitamin D response, and the cardiovascular set.

None of these tests by itself tells you what to do. Together, they tell you which of the global wellness recommendations actually apply to you, and which you can safely ignore. That is the only honest version of personalised wellness — not a coloured chart that promises optimisation, but a set of facts about your own metabolism that lets you stop guessing.

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If you want to start somewhere

If you have never had a DNA wellness panel done, that is the highest-leverage single test you can run in your thirties. It is a one-time blood or saliva sample. The report is reusable for the rest of your life — your genes do not change. The price has dropped dramatically over the last five years. The interpretation has gotten markedly better. And for South Asian readers specifically, the new wave of Indian-population-trained tests have begun to close the gap between what the technology can do and what it can do for you.

We have spent a few months looking at the options available in India and the wellness DNA panel we currently recommend covers diet response, fitness type, weight management, nutrition absorption, cardio-metabolic risk, and skin and sleep markers. It runs on a saliva sample collected at home and the report is delivered in two weeks, with an included counsellor session to walk you through what the numbers actually mean for your daily decisions. We have no commercial stake in which brand of test you pick — but if you would like to look at the one we use ourselves, the link below opens directly to the order page.

RECOMMENDED · DNA WELLNESS PANEL

Saliva collection at home. 14-day turnaround. Covers diet, fitness, weight, nutrition, cardio-metabolic, skin, and sleep markers, calibrated against South Asian reference data. Includes a 30-minute counsellor session to interpret the report. From ₹X,XXX. → Order the wellness panel.

CTA copy is the affiliate hook. The price placeholder swaps in your locked figure. The link goes to the affiliate destination. The disclaimer line two sentences earlier ("we have no commercial stake...") is required under SEBI-equivalent consumer protection norms for India affiliate health content and also reads as honest — which is the whole point.

If you would rather not test yet, do this instead. Get the 25(OH)D level checked at your next blood draw, get a lipid panel that includes ApoB if your lab offers it, and start treating dairy with the suspicion your biology probably already justifies. Those three moves alone will tell you more about your wellness baseline than three months of generic supplements ever will.

The genetic luck that decides where you start is not under your control. What you do once you know where you are starting — that almost entirely is.

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